

Is Physical Therapy Safe From AI?

The planning guide — the full scores by track, the six factors behind them, the honest trade-offs, and what to ask before you commit.



2.5

AI EXPOSURE · PHYSICAL THERAPY (HANDS-ON)

where 10 is most at risk

For anyone still choosing — read it straight through.



Before you read this

You're weighing a physical therapy or allied health degree — or a path into the field — and you want to know what that choice actually looks like in an AI economy. That's the whole job of this guide.

It's written to you directly, whether you are the one choosing or a parent thinking it through alongside them. Where it says "you," a parent can read "them"; it's the same decision from either side.

What this is. A facts document, not a recommendation. It sets out what's known, where the evidence is contested, and where we might be wrong — and then how to choose well given all of it.

If you're the parent, a note on how to use it. Don't lead with the scores. Open a conversation with a risk number and one of two things happens: they shut down, or they get defensive about a choice they're already leaning toward. Read it yourself first, then share it as information you both react to, not a verdict you've reached. They're choosing, not you — the job is to widen what's been considered, not to narrow it.

And one thing specific to this field, which matters more than anything else here. The AI question in physical therapy is about as settled as anything in this index. The financial question is not, and it is genuinely serious. The cost of a Doctor of Physical Therapy varies by more than \$300,000 between programs offering the identical qualification and leading to the identical career. That single fact should shape this conversation more than any score. If you read one section of this guide, read the financial question below.

The short answer

Safe — and it is barely a close call.

Hands-on physical therapy scores 2.5 out of 10 for AI exposure — the lowest score of any career in this index apart from surgery. Occupational therapy scores 2.6, speech-language pathology 3.1.

The risk question here is effectively settled. What isn't settled is whether the qualification is worth what it costs — and that depends almost entirely on which program you attend. Which is why which program you choose is most of the decision in front of you.

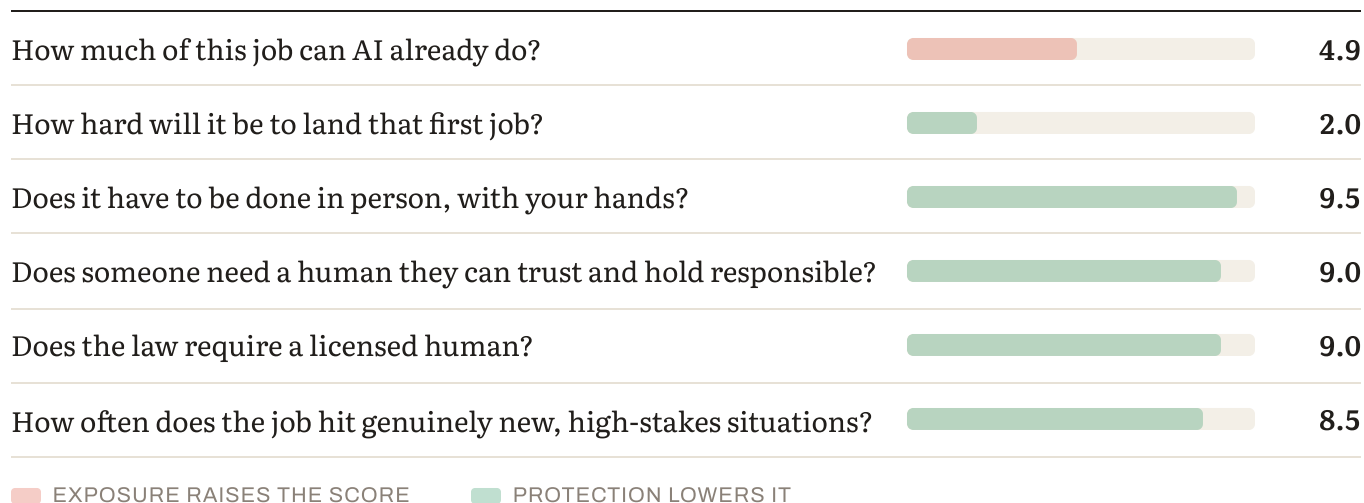
The scores

ROLE	2023	2025	FALL 2026	3-YR MOVEMENT	BAND
Physical therapy (hands-on)	2.0	2.2	2.5	+0.5	Very low
Occupational therapy	2.1	2.3	2.6	+0.5	Very low
Speech-language pathology	2.6	2.8	3.1	+0.5	Low
Respiratory therapy	2.6	2.8	3.1	+0.5	Low
Diagnostic imaging technologist	3.5	4.0	4.4	+0.9	Low–Moderate
Records / screen-based allied roles	5.1	5.6	6.1	+1.0	Moderate–High

Scores run 1–10, where 10 is most at risk. For context: the median profession in this edition sits around 5.5, surgery scores 2.1, bedside nursing 2.8, and entry-level software development 8.1.

Read the table this way: the +0.5 movement on hands-on therapy is entirely administrative — documentation and exercise-plan drafting. The clinical work has not moved at all. The spread inside the field is the thing to notice: the same qualification, moved behind a screen, roughly doubles in exposure.

Why it scores so low — the six factors



Here's how hands-on physical therapy answers them. Knowing *why* the clinical work scores 2.5 while the records-based roles sit at 6.1 is what tells you which version of the field to aim at.

HOW MUCH OF THIS JOB CAN AI ALREADY DO? (35% OF THE SCORE)

Less than almost any career we score.

Documentation and progress notes. Exercise plan drafting. Scheduling and billing. Outcome measure scoring. Insurance authorisation paperwork.

What resists: hands on a body, watching a gait in person, feeling resistance and guarding and compensation, motivating someone through months of painful work, and adjusting in real time to what a body actually does.

HOW HARD WILL IT BE TO LAND THAT FIRST JOB? (15%)

Strong protection. Licensure requires supervised clinical hours, which cannot be automated or offshored, and demand is growing well above average as populations age and survival after serious illness improves.

But there's a fork worth seeing now, and it isn't about finding a job — it's about which version, and at what cost. The clinical, hands-on roles are the protected ones; the records-centered and screen-based allied roles that score 6.1 are not, and a seat there is the exposed rung however healthcare-adjacent it looks. The way through is to enter on the clinical side — hands on a body, licensed, accountable — and to enter it through the least expensive accredited program you can, rather than taking on doctoral-scale debt for a screen-based role, or assuming the doctorate is the only door when the assistant route reaches the same protected work. A protected career entered at the wrong price is still a hard financial decision.

DOES IT HAVE TO BE DONE IN PERSON, WITH YOUR HANDS? (15%)

We rate this 9.5 — the highest physical rating we assign to any career. The protection-with-the-shortest-clock section below covers both why, and where we think it is most likely to change.

DOES SOMEONE NEED A HUMAN THEY CAN TRUST AND HOLD RESPONSIBLE? (15%)

Yes. Rehabilitation is a relationship over weeks and months, and a licensed clinician carries duty-of-care obligations.

DOES THE LAW REQUIRE A LICENSED HUMAN? (10%)

Comprehensively. Licensure, scope of practice and supervision requirements are written into statute.

HOW OFTEN DOES THE JOB HIT GENUINELY NEW, HIGH-STAKES SITUATIONS? (10%)

Frequently. Every body responds differently, and the judgment about when to push and when to stop is made in the moment.

The protection with the shortest clock

We give physical therapy 9.5 on the physical factor — our highest rating anywhere. It does most of the work in a score of 2.5, and it deserves examining honestly, because of our four protections, physical is the one we rank least durable over a long horizon.

Why it's so strong today. Therapy is manual by definition: hands on a body, resistance felt rather than measured, adjustment in real time to how a person actually moves. Robotics lags software badly, and lags furthest in exactly this setting — an unpredictable human body, in a room, over months.

Why we flag it anyway. Someone entering a doctoral program now will practice for forty years. Over that span, licensure and human trust move at the speed of law and society and are unlikely to shift much. Physical protection moves at the speed of robotics, which is the one thing actively targeting it.

And there is early evidence worth taking seriously. Studies of robotic-assisted rehabilitation report meaningfully better motor outcomes in some domains than conventional therapy — not merely comparable, better. If robotic delivery is superior for certain protocols, "hands-on work is safe" starts looking less like a wall and more like a line.

Two things keep our score where it is. Better outcomes *with* robotic assistance is not the same as therapy delivered *without* a therapist — in every deployment we found, a licensed clinician prescribes, supervises and adapts. And the peer-reviewed literature is explicit that translation into routine practice remains limited.

The specific transition to watch is the same pattern that splits the skilled trades: whether robotic systems move from *assisting* a therapist to *substituting* for therapist hours in structured, protocol-driven rehabilitation. Structured settings are where automation always arrives first. Unstructured ones — a stroke patient at home, an athlete with an atypical injury — are last.

The financial question — and it's the real one

This is the section that should shape the decision, and it has nothing to do with AI.

The debt is substantial and well documented.

- The typical DPT graduate carries roughly **\$140,000–\$170,000** in total student debt at graduation
- Starting salaries in outpatient settings run **\$75,000–\$95,000**
- That is a debt-to-first-year-salary ratio of roughly 1.7 to 2.0

For comparison, the same analysis puts new physicians at 0.6–0.9x (much higher debt, much higher salary), nurse practitioners at 0.9–1.3x, pharmacists at 1.5–1.8x, and master's-level occupational therapists at 1.4–1.7x.

The APTA's own research is blunt about the consequences. Asked to give the word they most associate with student debt, physical therapists most frequently answered "*burden*," "*unfair*," "*stressful*" and "*crushing*." Around 28% of PTs report holding additional paid employment alongside their primary position, up from 24% in 2017. Separate industry survey data finds 84% of those with PT debt feel their salary is insufficient to manage the payments.

And here is the number that should decide the program

Analysis across **206 DPT programs at 151 institutions** found debt-to-income ratios ranging from **0.82:1 at the best program to 4.42:1 at the worst**. The median sits at 1.8:1.

That is a difference of more than \$305,000 in total cost for the same degree, the same license and the same career.

Worse, the federal loan cap makes the gap unavoidable at expensive schools. The mean annual funding gap across all 206 programs is around **\$33,647** above the federal borrowing limit — which over three years means roughly \$95,000–\$101,000 in private borrowing at an average-cost school. Private loans carry higher rates and, critically, **do not qualify for income-driven repayment or Public Service Loan Forgiveness.**

The practical instruction is unusually simple. Attend the least expensive CAPTE-accredited program you get into. The difference in program quality is minor relative to the difference in tuition, and the financial consequence lasts decades.

The alternative almost nobody is told about

The physical therapist assistant route.

- **Two-year associate degree** rather than a bachelor's plus a three-year doctorate
- Median pay around **\$65,510**, against roughly \$80,000–\$90,000 for a PT
- Dramatically lower debt — around \$24,000 on average where it exists at all

The arithmetic is striking. A PTA working through the years a DPT student spends in graduate school could have earned close to \$290,000 by the time the therapist qualifies — while the therapist starts with \$140,000–\$170,000 of debt.

Over a full career the DPT wins, and comfortably: the income difference compounds across thirty years and the ceiling is considerably higher. But the gap is smaller than families assume, it takes many years to close, and it depends heavily on which program was attended.

And on our scoring, the AI exposure is similar — PTA work is hands-on, physical and protected by the same things that protect the therapist.

This is not an argument against the DPT. It's an argument that a student drawn to the hands-on work rather than to autonomy and diagnosis should know the assistant route exists before committing to the doctorate.

What the trend shows

Hands-on therapy: 2.0 → 2.2 → 2.5. Up 0.5 in three years — among the slowest movements in the index, and entirely administrative.

Records and screen-based allied roles: 5.1 → 5.6 → 6.1. Up 1.0.

The gap widened from 3.1 points to 3.6. The pattern holds exactly as everywhere else in this index: **move the same qualification behind a screen and exposure roughly doubles.** Which is the case for aiming at the clinical version deliberately, rather than drifting into the screen-based roles because they sound modern.

How durable is the protection?

PROTECTION	STRENGTH TODAY	DURABILITY	WHERE THE PRESSURE IS COMING FROM
Physical / embodied	Highest in the index	The one to watch	Robotic-assisted rehabilitation showing better outcomes in some protocols.
Regulatory / licensing	Strong	Most durable	Licensure and scope of practice move at the speed of legislation.
Human trust / accountability	Strong	Durable	Rehabilitation is a relationship sustained over months.
Judgment under novelty	Strong	Durable	Every body responds differently; the judgment is made in the moment.

The honest read: someone qualifying now will practice for forty years. Licensure and trust should hold comfortably. **Physical protection is the strongest shield today and the one with the most credible long-term threat — and it is worth watching structured, protocol-driven rehabilitation specifically rather than the profession as a whole.**

How the role will actually change

WAS THE JOB	IS BECOMING THE JOB
Writing progress notes after every session	Reviewing a generated draft
Building the exercise program by hand	Adapting a generated program to this patient
Scoring outcome measures	Reading outcome data that arrives scored
Chasing insurance authorisation	Largely automated
Hands on a body	Unchanged
Knowing when to push and when to stop	Unchanged

The pattern here is unusually clean. In physical therapy the automatable layer and the clinical layer are cleanly separable, and what's automating is documentation — which the APTA's own research names among the primary contributors to burnout.

Human strengths that will matter most

1. **Manual assessment and treatment** — feeling resistance, guarding and compensation. The

least automatable thing in this index.

2. **Motivating someone through months of painful work** — most of rehabilitation is adherence,

and adherence is a relationship.

3. **Real-time judgment** — knowing when to push and when to stop, made in the moment with a

body in front of you.

4. **Accountability** — carrying duty of care for someone's recovery. 5. **Verification** — knowing when a generated program or note doesn't fit this patient.

Notably absent: protocol recall and documentation speed. These were traditional markers of a strong new graduate and are the fastest-depreciating.

And underneath all five, two traits the score can't grade: **curiosity and adaptability**. The specific tools — the documentation system, the outcome-data platform, the robotic and sensor-assisted rehabilitation rigs — will turn over many times across a career. The clinicians who compound are the ones who stay genuinely curious about how a body recovers and re-tool fastest, rather than defending the one way of working they trained on. In a field where the physical protection is real today but has the shortest clock, that flexibility is itself a form of protection — and understanding where the new technology helps and where it misleads is a competency almost nobody is teaching yet.

Other things to consider about this job

What's genuinely good about it

Demand is strong and demographically driven rather than cyclical. Physical therapists are projected to grow around 11% and occupational therapists 14% through 2034 — both far faster than average — driven by an ageing population and improving survival after serious illness.

What therapists report as rewarding is unusually consistent:

- **Visible progress** — you watch someone recover across weeks in a way few jobs offer
- **Relationships that run over months**, not minutes
- **Autonomy**, with private practice a realistic destination
- **Physical, active work** rather than a desk
- **Genuine variety** across settings — hospital, outpatient, sports, paediatric, home health, neurological

And the career is sustainable into later life with specialization, and portable across settings and geographies.

What's genuinely hard about it

The debt, covered in the financial section above, and it is the dominant issue.

Burnout is high and the causes are not what you'd expect. Around 70% of physical therapists report moderate to high burnout, and the most-cited drivers are unrealistic productivity

demands, heavy documentation requirements and administrative load — not the clinical work itself.

That's worth noticing, because it's precisely the layer that AI is removing. This may be the career in the whole index where automation most directly addresses the profession's actual complaint.

The work is physically demanding over decades. Standing, lifting, transferring patients.

And some patients do not improve, which is harder to sit with than students expect.

Who this work suits — and who it doesn't

Physical therapy tends to suit people who:

- **Want to see progress they can measure** — the single most-cited satisfaction
- **Are comfortable with sustained physical contact** and with bodies generally
- **Can motivate people who don't want to be motivated** — most of the job is adherence
- **Prefer moving to sitting**
- **Are patient with slow improvement**

It's harder for people who:

- **Need quick resolution** — rehabilitation is measured in months
- Find **repetitive documentation** draining, though this is the automating part
- Want **high early earnings** relative to the training cost
- Are uncomfortable when **someone doesn't get better**

What else is moving this market

Demographics are the dominant force, and they are durable rather than cyclical.

Non-opioid pain management has increased referrals to physical therapy meaningfully — a policy shift rather than a technological one.

Productivity pressure is the profession's structural complaint, particularly in outpatient settings where reimbursement models push toward higher patient volumes per therapist.

And employer loan assistance is emerging but rare — around 80% of PTs and OTs carry student debt while only about 8% receive any employer repayment support. Where it exists it is

worth several years off a payoff timeline, and it is a legitimate thing to ask about at interview.

The AI-native advantage — what to actually do about it

The situation: AI in physical therapy is arriving as documentation relief rather than clinical substitution, which makes this one of the more straightforwardly positive pictures in the index. A good program should be building this fluency deliberately, not banning the tools or ignoring them.

WHAT AN AI-NATIVE THERAPIST LOOKS LIKE

- 1. Reclaim the time deliberately.** Documentation automation only helps if the hours saved go somewhere. Therapists who are intentional about this get more patient contact; those who aren't watch productivity targets absorb it.
- 2. Verify.** Knowing when a generated note or program doesn't fit this patient — a clinical skill rather than a technical one.
- 3. Work with robotic and sensor-assisted rehabilitation.** These systems are arriving. Understanding where they help and where they mislead is a genuinely new competency and almost nobody is teaching it.
- 4. Use outcome data properly.** Continuous movement and adherence data is becoming available. Interpreting it clinically is a real skill.
- 5. Contribute to how it's implemented.** Services are writing these policies now, largely without practitioner input.

CONCRETE PREPARATION

Before the program: compare programs on total cost, not reputation. This is the single highest-value action available and it is worth more than anything else in this section.

During: get clinical exposure across as many settings as possible — the setting shapes the career more than the qualification does. Take manual therapy and assessment seriously; that is the protected core.

Entering: ask about employer loan repayment assistance, and about productivity expectations specifically. Both shape the first five years more than the job title does. The sections below cover exactly what to ask.

The routes in

ROUTE	TYPICAL LENGTH	NOTES
DPT (Doctor of Physical Therapy)	3 years post-bachelor's	The standard route. Cost varies by over \$300,000 between programs.
Physical therapist assistant	2-year associate	Hands-on work, far lower debt, lower ceiling. Genuinely under-considered.
Occupational therapy	Master's or doctorate	Similar protection, focused on daily function rather than movement.
Speech-language pathology	Master's	Consistently under-recruited, scores 3.1.
Respiratory therapy	Associate or bachelor's	Shorter route, acute-care focused, scores 3.1.
OT assistant	2-year associate	Strong projected growth, short training.

Where these qualifications can lead later

Specialization (orthopaedic, neurological, paediatric, sports, geriatric), private practice ownership, clinical education and academia, research, healthcare management, ergonomics and occupational health, and industry roles in devices and rehabilitation technology.

The pattern in this index holds: the further from hands-on patient contact, the higher the exposure. Records-centered and utilization roles score 6.1 against clinical work at 2.5. That's the whole map, and it's worth having in mind when a program describes where its graduates end up.

What to look for in a program

1. Total cost — and calculate the debt-to-income ratio yourself. This matters more than anything else on this list, and it is the thing prospectuses discuss least.

2. CAPTE accreditation. Non-negotiable.

3. Breadth of clinical placements. How many settings, and which? Setting shapes career satisfaction more than the qualification does.

4. Licensure exam pass rates.

5. Does the curriculum address rehabilitation technology and AI-assisted documentation?

Questions to ask a program

On cost — ask these first:

- What is the total cost of attendance, including living expenses, for the full program?
- What is the median debt at graduation for your graduates?
- What proportion of students need private loans beyond the federal cap?

On clinical experience:

- How many weeks of clinical placement, across how many distinct settings?
- Are placements guaranteed, and how are they allocated?

On outcomes:

- What's your licensure exam first-time pass rate?
- Where are graduates working at one year, and in what settings?
- What's your attrition rate?

On technology:

- How does the curriculum address robotic-assisted rehabilitation and AI documentation tools?

Red flags: evasiveness about total cost; placements not guaranteed; no data on graduate debt; a curriculum unchanged since 2022.

The career-hunting checklist

Choosing well is step one. This is what turns a good choice into a good outcome once you're in — worth reading now, because the best time to pick a program is knowing exactly what you'll need to do with it. The first list is for the degree and training years; the second is for the job hunt that follows.

During the degree / training — build what can't be automated

- **Protect the clinical core.** Manual therapy and assessment — feeling resistance, guarding and compensation — is the 2.5 work. The records-centered and screen-based allied roles are the 6.1. Take the hands-on modules as seriously as the score says they deserve.
- **Attend the least expensive CAPTE-accredited program, and minimize private borrowing.** The federal loan cap leaves a mean funding gap of around \$33,647 a year at an average-cost school — roughly \$95,000–\$101,000 in private loans over three years, and private loans don't qualify for income-driven repayment or Public Service Loan Forgiveness. Cost is the decision here, not reputation.
- **Get clinical breadth across as many settings as possible.** Hospital, outpatient, sports, paediatric, home health, neurological — the setting shapes the career more than the qualification does.
- **Take rehabilitation technology seriously.** Robotic and sensor-assisted rehabilitation and continuous outcome data are arriving; knowing where they help and where they mislead is the new competency, and almost nobody is teaching it.
- **Get fluent with AI documentation as a clinician, not a shortcut.** Be able to verify — to know when a generated note or program doesn't fit this patient. That is a clinical skill, and it's the one that survives.
- **Feed curiosity and adaptability on purpose.** The tools will turn over repeatedly; the clinician who re-tools fastest, and stays genuinely curious about how a body recovers, compounds.
- **Keep the frame wide.** Properly vet the assistant routes (PTA, OTA), occupational therapy, speech-language pathology and respiratory therapy before ruling any of them out — several reach the same protected work at a fraction of the debt.

During the job hunt — aim, don't just apply

- **Target the clinical, hands-on lanes on purpose.** Direct patient care in PT, OT, SLP or respiratory therapy — not records-centered or utilization roles, which are the 6.1 rung however healthcare-adjacent they look.

- **Read the role, not just the title.** A screen-based "records" or "utilization" allied role is the automating layer (6.1). If you take one, take it as a deliberate stepping-stone with an exit in mind, not as the plan.
 - **Don't grab a role just because it's on offer in an exposed lane.** A role being available doesn't make it safe. An employer still building out a large screen-based allied or records team may simply be behind the curve — and that's exactly the rung that gets cut first once they catch up. Availability can be a lagging indicator, not a green light.
 - **Ask about productivity expectations and employer loan repayment assistance before accepting.** Only about 8% of PTs and OTs get any repayment support, but where it exists it's worth several years off the payoff — and both these things shape the first five years more than the title does.
 - **Prefer the setting and seat with more patient contact.** Around 70% of therapists report moderate to high burnout, driven by productivity quotas and documentation rather than the clinical work — so weigh a lower-quota clinical seat over a documentation-heavy outpatient mill.
 - **Ask the employer where AI and rehabilitation technology are heading in the role.** Whether they're building the job around documentation relief and robotic-assisted rehab or waiting to be forced to tells you if the seat grows or is quietly being hollowed out.
 - **Weigh the trajectory, and the debt math, over the starting salary.** The financial decision in this field is made program-by-program and role-by-role, not by chasing the highest first-year number.
-

Go deeper — further reading

- **Physical Therapists — Occupational Outlook Handbook** — US Bureau of Labor Statistics. Primary source and free: projected growth, median pay, entry requirements. Read the **Physical Therapist Assistants** page alongside it — the comparison is the most useful twenty minutes available on this decision.
- **Impact of Student Debt on the Physical Therapy Profession** — American Physical Therapy Association. The profession's own research into what the debt is doing to its members, including the finding that 28% hold additional employment. Read it before committing to an expensive program.
- **Is physical therapy worth the debt? 2026 ROI analysis** — the program-by-program debt-to-income analysis across 206 DPT programs. If you read one thing before choosing where to apply, read this.

The counterargument — read this one:

The strongest case against our 2.5 isn't that AI will replace therapists. It's that a very low AI score may lead a family to under-weight a genuinely serious financial risk.

On this reading, our framework tells a student that physical therapy is among the safest careers available — which is true — while saying nothing about a debt-to-income ratio that can reach 4.42:1 at the wrong program, private borrowing that doesn't qualify for forgiveness, and 84% of indebted PTs reporting their salary is insufficient to manage the payments.

A career can be automation-proof and still be a poor financial decision.

Where we land: this is entirely fair, and it's why the financial section sits before the "other considerations" section rather than inside it. Our score measures automation exposure only. For physical therapy specifically, the AI answer is genuinely reassuring and the financial answer depends almost entirely on which program someone attends — and conflating the two would be the most damaging thing this document could do.

Bottom line

On AI exposure, this is about as good as it gets. Hands-on physical therapy scores 2.5 — lower than anything in this index except surgery — protected by manual work, licensure, trust and genuine unpredictability. Demand is demographically driven and durable. And the automation that is arriving removes documentation, which the profession's own research names among the leading causes of burnout.

The real decision is financial, and it is program-specific to an extraordinary degree. Debt-to-income ratios across 206 DPT programs run from 0.82:1 to 4.42:1 — a spread of over \$305,000 in total cost for the same degree and the same career.

So the useful advice is unusually concrete: attend the least expensive accredited program you get into, because the quality difference between programs is small relative to the cost difference. Minimize undergraduate debt first. Ask employers about loan repayment assistance. And if the pull is hands-on rehabilitation rather than autonomy and diagnosis, look seriously at the assistant route before committing to the doctorate.

And one thing worth saying that no score captures. Watching someone walk again, or lift their arm again, or return to something they thought they'd lost — very few jobs offer that at all. The financial caution above is real and it is about *which program*, not about *whether*.

Questions worth sitting with

Whether you're the one choosing or a parent thinking it through together, these are the conversations that lead to a better decision. Read "you" as whoever the choice is about.

About the work itself

1. What made physical therapy appealing? Push past "I want to help people" to something specific — an injury, a person, a moment. 2. Are you comfortable with sustained physical contact, and with bodies generally? 3. Can you motivate someone who doesn't want to be motivated? Most of rehab is adherence. 4. How would you cope with a patient who doesn't improve?

Reacting to the findings

5. Hands-on therapy scores 2.5 and records-based allied work 6.1. Does the setting matter to you? 6. The guide flags physical protection as the strongest today and the least durable long-term. What do you make of that? 7. Around 70% of PTs report moderate to high burnout, driven by documentation and productivity demands rather than clinical work. Did that surprise you?

The money conversation, and this is the important one

8. **Have you calculated the debt-to-income ratio for each program you're considering?** Total cost of attendance divided by realistic starting salary. Do it before applications, not after offers. 9. Do you understand that the same degree costs over \$300,000 more at some programs than others? 10. Do you know that borrowing above the federal cap means private loans, which don't qualify for income-driven repayment or forgiveness? 11. **Have you looked at the physical therapist assistant route?** Two years, far less debt, similar AI protection, lower ceiling.

Testing it against reality

12. **The most valuable single action:** shadow a therapist in two different settings — outpatient orthopaedic and something else entirely, like neurological or paediatric. The settings are very different jobs. 13. Find someone five years post-qualification and ask directly about their loan payments and whether they'd choose the same program.

Widening the frame

14. Have you considered **occupational therapy, speech-language pathology or respiratory therapy**? All score similarly well, and speech-language pathology is consistently under-recruited. 15. What about **nursing**? Scores 2.8, reaches licensed practice in two to four years, and costs a fraction as much. If none of these were available, what would you choose? The answer usually reveals what you're drawn to.

This is analysis and informed judgment about a fast-moving situation. For physical therapy specifically, we've flagged that the profession's most serious risk is financial rather than technological, and that it varies enormously by program — something our six factors cannot see.

Scores for 2023 and 2025 are retrospectively calculated using the current methodology. Scores measure exposure to what AI can already do — not how much any particular employer has deployed.

Technical scoring appendix — Physical Therapy & Allied Health

Pivotum Degree Risk Index — Fall 2026 Edition

This appendix is the audit trail. It sets out the formula, the rating anchors, every factor rating behind every track score in this profile, the backcast arithmetic, a sensitivity analysis, and the specific things that would change our mind.

We publish it because a score nobody can check is an opinion with a decimal point.

1. The formula

$$\begin{aligned} \text{Risk} = & 0.35 \times \text{Automatability} \\ & + 0.15 \times \text{EntryErosion} \\ & + 0.15 \times (10 - \text{Physical}) \\ & + 0.15 \times (10 - \text{Trust}) \\ & + 0.10 \times (10 - \text{Regulatory}) \\ & + 0.10 \times (10 - \text{Judgment}) \end{aligned}$$

Two exposure factors carry 50% of the weight; three protection factors carry 40%; judgment under novelty carries the remaining 10% and sits on the protection side. The deliberate consequence is that protection can outweigh exposure — which is why a highly automatable job like teaching still scores low.

Bands. Very low below 3.0 · Low 3.0–4.4 · Moderate 4.5–5.4 · Moderate–High 5.5–6.9 · High 7.0 and above.

2. Rating anchors

HOW THE 0–10 RAW RATINGS ARE ANCHORED

Every factor is rated on its own 0–10 scale before weighting. The anchors are identical across all twenty-eight careers, which is what makes the scores comparable.

Task automatability (A) — *what share of the working week could a capable current system already do to an acceptable standard?*

RATING	ANCHOR
0–2	Almost nothing. The work is physical, relational or wholly novel.
3–4	Administrative surround only — notes, scheduling, correspondence.
5–6	A meaningful minority of the week, mostly documentation and lookup.
7–8	A large share, including tasks central to the junior version of the role.
9–10	Most of the described duties, at or near professional standard.

Entry-path erosion (E) — *how much has the traditional route into this work narrowed?*

RATING	ANCHOR
0–2	Protected by statute or physical necessity. Training hours cannot be automated.
3–4	Stable. Some junior tasks absorbed; the apprenticeship survives.
5–6	Visible narrowing. Employers prefer experience; junior intake reduced.
7–8	Substantial. The work juniors learned on has largely gone.
9–10	The training layer and the automatable layer were the same layer.

Physical / embodied (P) — *must this be done in person, with a body, in an unpredictable setting?*

RATING	ANCHOR
0–2	Fully screen-based. Location-independent.
3–4	Occasional presence; the core work is not physical.
5–6	Regular physical presence, in largely controlled conditions.
7–8	Substantially physical, with some variability of setting.
9–10	Irreducibly physical, in conditions that differ every time.

Human trust / accountability (T) — *does someone need a specific human they can rely on, and must a human answer when it goes wrong?*

RATING	ANCHOR
0–2	Output is anonymous and reviewed by others.
3–4	Work is signed off by someone else. No personal reliance.
5–6	Some client relationship; accountability is institutional.
7–8	Named responsibility, with commercial or professional consequence.
9–10	The relationship is the service, and liability attaches personally.

Regulatory / licensing (R) — *does the law reserve this act to a qualified human?*

RATING	ANCHOR
0–2	No license exists and none is proposed.
3–4	Certification is customary but not reserved.
5–6	Partial reservation, varying by jurisdiction or task.
7–8	Licensed practice, with some unreserved adjacent work.
9–10	Comprehensive statutory reservation, including of the training route.

Judgment under novelty (J) — *how often does the work present something with no matching precedent, where being wrong is costly?*

RATING	ANCHOR
0–2	Routine by design. Deviation is an error, not a case.
3–4	Occasional exceptions, handled by escalation.
5–6	Regular non-standard cases within a known range.
7–8	Frequent genuine novelty with material consequences.
9–10	Novelty is constant, and in some fields adversarially generated.

3. Factor ratings by track

Ratings are the Fall 2026 edition unless a range is shown. **P, T, R and J are held constant across editions** — those protections are structural and do not move on a three-year horizon. All measured movement is in A and E.

PHYSICAL THERAPY (HANDS-ON) — 2.5 (VERY LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.1	4.5	5.2	1.82
Entry-path erosion	15%	1.2	1.5	1.8	0.27
Physical / embodied	15%	9.5	9.5	9.5	0.07
Human trust / accountability	15%	9.3	9.3	9.3	0.1
Regulatory / licensing	10%	9.3	9.3	9.3	0.07
Judgment under novelty	10%	8.5	8.5	8.5	0.15
				Total	2.49 → 2.5

OCCUPATIONAL THERAPY — 2.6 (VERY LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.3	4.7	5.5	1.92
Entry-path erosion	15%	1.2	1.5	1.8	0.27
Physical / embodied	15%	9.4	9.4	9.4	0.09
Human trust / accountability	15%	9.3	9.3	9.3	0.1
Regulatory / licensing	10%	9.3	9.3	9.3	0.07
Judgment under novelty	10%	8.5	8.5	8.5	0.15
				Total	2.61 → 2.6

SPEECH-LANGUAGE PATHOLOGY — 3.1 (LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	5.2	5.7	6.4	2.24
Entry-path erosion	15%	1.5	1.8	2.0	0.3
Physical / embodied	15%	8.5	8.5	8.5	0.22
Human trust / accountability	15%	9.3	9.3	9.3	0.1
Regulatory / licensing	10%	9.3	9.3	9.3	0.07
Judgment under novelty	10%	8.5	8.5	8.5	0.15
				Total	3.09 → 3.1

RESPIRATORY THERAPY — 3.1 (LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	5.1	5.6	6.3	2.2
Entry-path erosion	15%	1.5	1.8	2.0	0.3
Physical / embodied	15%	8.8	8.8	8.8	0.18
Human trust / accountability	15%	9.0	9.0	9.0	0.15
Regulatory / licensing	10%	9.3	9.3	9.3	0.07
Judgment under novelty	10%	8.2	8.2	8.2	0.18
				Total	3.08 → 3.1

DIAGNOSTIC IMAGING TECHNOLOGIST — 4.4 (LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.9	6.1	7.0	2.45
Entry-path erosion	15%	2.5	3.0	3.4	0.51
Physical / embodied	15%	7.5	7.5	7.5	0.38
Human trust / accountability	15%	7.0	7.0	7.0	0.45
Regulatory / licensing	10%	8.5	8.5	8.5	0.15
Judgment under novelty	10%	5.5	5.5	5.5	0.45
				Total	4.38 → 4.4

RECORDS / SCREEN-BASED ALLIED ROLES — 6.1 (MODERATE-HIGH)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.6	5.9	7.1	2.48
Entry-path erosion	15%	4.5	5.0	5.5	0.82
Physical / embodied	15%	1.5	1.5	1.5	1.27
Human trust / accountability	15%	5.5	5.5	5.5	0.67
Regulatory / licensing	10%	7.0	7.0	7.0	0.3
Judgment under novelty	10%	4.5	4.5	4.5	0.55
				Total	6.11 → 6.1

4. Backcast arithmetic

Worked example for the headline track, showing exactly how the three-year movement is composed.

Physical therapy (hands-on)

EDITION	A	E	PROTECTION DEFICIT (FIXED)	SCORE
2023	4.1	1.2	0.4	2.0
2025	4.5	1.5	0.4	2.2
Fall 2026	5.2	1.8	0.4	2.5

Movement decomposition: automatability contributed **+0.39** and entry-path erosion **+0.09**, for a total of **+0.5** points over three years.

The 2023 and 2025 figures are retrospective reconstructions using the current methodology, not archived scores from published editions. They are our best assessment of what each factor would have rated at the time, given what was then demonstrable. They are not measurements.

5. Sensitivity

How far the score moves if a single factor is rated one point differently:

FACTOR	±1 POINT MOVES THE SCORE BY
Task automatability	±0.35
Entry-path erosion	±0.15
Physical / embodied	±0.15
Human trust / accountability	±0.15
Regulatory / licensing	±0.10
Judgment under novelty	±0.10

What this means in practice. A one-point disagreement about automatability moves a score by 0.35 — enough to shift a track between bands at the margins. A one-point disagreement about licensing moves it by 0.10, which almost never changes a band.

So if you disagree with one of our numbers, the automatability rating is where the disagreement matters most, and it is the rating we would most want challenged.

6. Stated limitations

We measure capability, not deployment. Scores reflect exposure to what AI can already do at the leading edge — not how much any particular employer has adopted. Adoption lags capability by years and lags unevenly: large, well-funded organisations first; small, rural, public-sector and thin-margin employers much later. The lag is a buffer, not a shelter. It buys time without changing direction.

We do not measure demand. A task can be highly automatable and highly in demand simultaneously. Where the labor market and our framework disagree, we say so in the profile rather than adjusting the score to fit.

We do not measure oversupply. The number of people entering a field is outside our six factors, and for some careers it matters more than automation.

We do not measure industry economics. A profession can contract for reasons entirely unrelated to technology.

We do not measure whether the work is worth doing. Pay, satisfaction, debt, hours and meaning sit outside the score and are covered separately in each profile — often at greater length, because for several careers they matter more.

Sub-track definitions are ours. Job titles vary between employers and countries. We have chosen tracks that reflect genuinely different work, not official classifications.

7. What would change our mind

Each edition names specific, checkable indicators. If these move, the scores move — including downward.

Across the index:

- **Graduate hiring share.** If employers in the professions where we rate entry-path erosion highly return to hiring juniors at pre-2023 rates, those ratings fall.
- **Content of mandated training hours.** Several professions protect their on-ramp through required supervised experience. If the content of those hours shifts from producing work to supervising it, the protection weakens without any rule changing.
- **Offsite and prefabricated construction share.** The clearest test of our physical-protection ratings: automation performs well in controlled settings, so moving work indoors raises exposure without any robotics breakthrough.

- **Verified deployment rather than capability.** If adoption stalls materially below capability for several editions, our leading-indicator approach is overstating near-term risk and we will say so.

We will report movement in either direction. A framework that only ever revises upward is not measuring anything.

Scores measure exposure to what AI can already do — not how much any particular employer has deployed. The 2023 and 2025 figures are retrospective reconstructions using the current methodology.